



Sullivan and the intersubjective perspective

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ORIGINAL ARTICLE

Sullivan and the intersubjective perspective

MARCO CONCI

Abstract

The author sees H.S. Sullivan's (1892–1949) interpersonal theory as the best theoretical framework for the contemporary intersubjective perspective in psychoanalysis and presents the former in its pluridimensional articulation. After having extended Freud's therapeutic approach to psychotic patients, Sullivan developed both a developmental psychology and a psychoanalytic and psychotherapeutic technique based on the "interpersonal field" as the basic unit of study. To the pluridimensional character of his theory also belongs its application to the cultural and social aspects of our personal identity. Having the contemporary psychoanalytic authors who shaped the intersubjective perspective limited themselves to the clinical dimension, Sullivan's interpersonal theory can still provide the theoretical framework that any psychoanalytic perspective needs.

Key words: *H.S. Sullivan, interpersonal psychiatry and psychoanalysis, psychotic patients, countertransference, interpersonal field, participant observation, developmental psychology, pluridimensional frame, intersubjectivity*

In his 1995 paper "Intersubjectivity in psychoanalysis: A critical review," Jonathan Dunn described the intersubjective perspective as centered around the definition of mind as "striving for connection and communication, rather than discharge and gratification of endogenous instinctual pressures" (1995, p. 724). Following such a metapsychological point of view, intersubjective analysts "see the clinician and the patient co-constructing the clinical data from the interaction of both members' particular psychic qualities and subjective realities" (Dunn, 1995, pp. 723–4). The author furthermore retraces the emergence of the intersubjective paradigm in psychoanalysis to the critique undergone by Freud's concepts of countertransference and of ego function. Although Dunn cites a large number of authors, mostly belonging to the self-psychological and to the interpersonal and relational groups, he does not mention the name and the work of what I consider to be the pioneer of the intersubjective turn in psychoanalysis, that is Harry Stack Sullivan (1892–1949). Since Sullivan's work and legacy are not well enough known, I will take them up again in

this paper, in order to show how his point of view can still represent the best theoretical framework for the intersubjective perspective.

Sullivan is one of my favorite psychoanalytic authors. After teaching a course on his life and work at the University of Venice at the beginning of 1988, I met the late Stephen Mitchell in Florence on his first trip to Italy with Jay Greenberg, to give lectures and seminars around their book *Object relations in psychoanalytic theory* (Greenberg & Mitchell, 1983) at what later became the Istituto Sullivan, and discovered that Mitchell was also very fond of Sullivan's work. With his encouragement, and thanks to the contribution given by my supervisor Gaetano Benedetti to the introduction of Sullivan's work into Italy (Benedetti, 1961), I started working on a book on Sullivan's life and work, which was published in Italy in 2000 and in Germany in 2005. Its American edition, *Sullivan revisited – Life and work. Harry Stack Sullivan's relevance for contemporary psychiatry, psychotherapy and psychoanalysis* is, at the time of writing this article, now in print, and will soon be able to be ordered by simply Googling Sullivan's name. Sullivan went beyond Freud not only by extending the latter's new therapeutic approach to psychotic patients, but also by formulating a new clinical point of view, based upon the interaction of analyst and patient.

The aim of this paper is to let you hear Sullivan's voice, to introduce you to his fascinating system of

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thought, and to show its pluridimensional character. By pluridimensional character, I mean the fact that Sullivan's work includes not only the clinical and the developmental, but also the empirical and the applied or interdisciplinary dimension of psychoanalysis. From this point of view, the pluridimensional character of Freud's own work was certainly an important influence on Sullivan. An essential aspect of such an orientation is also his emphasis on the cultural and social coordinates of our personal identity. This is how Sullivan's interpersonal point of view can still represent the best theoretical framework of the intersubjective perspective – as I defined it above, following Jonathan Dunn.

Sullivan's voice

Sullivan's first ground-breaking contribution, which started in the 1920s and is documented in detail in the anthology *Schizophrenia as a human process* edited by Helen Swick Perry in 1962, was to show how wrong Freud was in distinguishing patients into those capable and those incapable of transference. Parallel to such an important paradigmatic change is his critique of Emil Kraepelin's (1856–1926) concept of schizophrenia as being a degenerative brain condition and not a psychological illness – as Jay Greenberg and Stephen Mitchell remind the reader in their above-mentioned book (Greenberg & Mitchell, 1983, p. 83). Although he seldom mentioned the word “countertransference,” Sullivan based his therapeutic work with the sickest patients on his “participation” in their condition, that is, on the subjective resonances evoked by them in him. This was the basis of his therapeutic attitude. He thus went beyond Freud's original and narrow use of the concept, exclusively related to the neurotic component of our reaction to patients, and pioneered the definition of “countertransference” later introduced by Paula Heimann in the 1950s. Here is what Paula Heimann wrote in 1950: “the analyst's emotional response to his patient within the analytic situation represents one of the most important tools for his work. The analyst's countertransference is an instrument of research into the patient's unconscious” (1950, p. 81).

Indeed, the experience and resolution of the dissociative episode that his biographer Helen Swick Perry documented as having taken place in his life between the spring of 1909 and the fall of 1911 must have allowed Sullivan to much more easily attune himself to the inner world of psychotic patients than the average psychiatrist and psychoanalyst (Swick Perry, 1982). From such a line of work also emerged the interpersonal point of view, whose systematic

formulation he developed in the 1940s and which can be found in his book *Interpersonal theory of psychiatry* (1953). In other words, as Freud had used his own psychopathology – which he got in touch with and analyzed in his letters to Wilhelm Fliess – as a key to the therapy that he developed with neurotic patients, so also did Sullivan do this with his psychotic patients. A further similarity between Sullivan and Freud has of course to do with the autodidactic character of the development of their theories, an aspect that I have repeatedly emphasized in my above-mentioned book on Sullivan (Conci, 2010).

But let us now turn to Sullivan, and to his peculiar voice, that is, to how he talked about psychotic patients on the basis of his own personal and professional experience, as he did for example when he presented his clinical work to the social scientists participating in the first of the two Colloquia on Personality Investigation, which he organized with W.A. White (1870–1937) in 1928 and 1929:

Aside from perhaps the most significant of my activities – the attending of conferences with the social scientists – my attempts at collecting data on personality take the form of living with schizophrenic individuals. In the address from the chair, it was stated that general familiarity with the work of each other would be assumed. As, however, there seems to be no particular consensus even among my psychiatric colleagues as to the connotation of the term schizophrenia, it is perhaps rather of interest of this conference that I say something by way of explanation.

... Some schizophrenics impressed me, years ago, as singularly interesting [says Sullivan after having first presented their clinical picture] in part, because of their striking manifestations of some of my own most highly esteemed traits. I refer, of course, to my personal appraisal of my traits. I therefore cultivated them and this gave such complimentary returns to my self-respect and feeling of well-being that I have continued it since, alternating chiefly with attendance upon scientific deliberations and the like.

I find many such individuals very human indeed, particularly when they have not been exposed for a long period to a ‘good’ psychiatric care. It has seemed to me that in these schizophrenics one finds in almost laboratory simplicity the manifestations of complex processes which are combined in the more fortunate of us in such great complexity that they can scarcely be grasped and subjected to anything approximating scientific critique. ... One discovers on acquaintance with these individuals something different indeed from the traditional psychiatric picture ... (1928/1962, pp. 218, 220)

As Jung, working with schizophrenic patients, had ended up renouncing to Freud's libido concept, so Sullivan ended up considering schizophrenia what we can call a “personal sickness,” that is, the sickness

developed by an individual who does not succeed in becoming a person. I am of course making reference to those individuals who cannot easily distinguish between internal and external reality, and who either retreat silently into themselves or develop a so-called “productive” symptomatology, with hallucinations and delusions. It was in this context of discovery that Sullivan had for the first time utilized the word “interpersonal” in 1927, at the end of the first section of the paper “The onset of schizophrenia”:

Study of the onset of the disorder in male patients of this hospital seems to establish two factors preliminary to schizophrenic psychoses. Firstly, the appearance of the disorder is late in a long series of subjectively difficult adjustive efforts. Secondly, it seems never to occur in those who have achieved if only for a short time a definitely satisfying adjustment to a sex object. We have not been successful in our effort to identify exactly the factors which cause milder maladjustive efforts to pass over into schizophrenia. Neither do we believe we are justified by accumulated facts, to stress the sex factor as of exclusive importance. Much more data is needed in regard to the onset of the malady; at this stage, however, there seems little reason to doubt that cultural distortions provided by the home are of prime importance. We have not seen maladjustment which was without a foundation of erroneous attitudes which parents or their equivalent had thrust upon the child. We have found all sorts of maladjustment in the history of patients who suffered the grave psychosis, but regardless of vicious influences subsequently encountered, the sufferer had acquired the tendency to such an illness while in the home situation. *Interpersonal factors seem the effective elements in the psychiatry of schizophrenia.* (1962, p. 104, emphasis added)

Published in 1962 in the anthology *Schizophrenia as a human process*, this paper represents one of the turning points of Sullivan’s overall work. As I wrote in the preface to the Italian edition of this anthology (which came out in 1993 under the title *Scritti sulla schizofrenia* [Papers on schizophrenia]), we can at best understand Sullivan’s interpersonal theory of psychiatry of the 1940s as his attempt to provide an answer to the questions that had arisen in his work on schizophrenia in the 1920s and 1930s, with particular regard to the subtle and complex interpersonal processes that enable a newborn infant to become a healthy human being.

Before taking up some of the key concepts of Sullivan’s interpersonal theory, let me propose to you some of the basic conclusions to which he came around 1930, at the end of his first research journey into the nature and treatment of schizophrenia, which he conducted between the fall of 1922 and the summer of 1930 at the Sheppard Pratt Hospital in Maryland.

First:

The only tools that have shown results that justify any enthusiasm in regard of the treatment of schizophrenia are the *psychoanalytic* procedures and the *socio-psychiatric* program which the writer evolved from them. (Sullivan, 1962, p. 283, original emphasis)

Second:

In conclusion, it may be restated that, at least in the case of the male, fairly young schizophrenic patients whose divorcement from fairly conventional behavior and thought has been rather abrupt, when received under care before they have progressed either into hebephrenic dilapidation or durable paranoid maladjustments, are to be regarded as of good outlook for recovery and improvement of personality, if they can be treated firstly to the end of socialization, and thereafter by more fundamental reorganization of personality. (Sullivan, 1962, p. 290)

Eighty years after its originary formulation, such a (very good) therapeutic program has still found so comparatively little diffusion that it makes even more sense to keep it in mind.

Sullivan’s field concept and his interpersonal theory

No wonder that the most well-known of Sullivan’s posthumous works, *Interpersonal theory of psychiatry* (1953), is based on another new important concept pioneered by him, the concept of “interpersonal field,” around which his own definition of the field of psychiatry is centered. Psychiatry “arises not from a special kind of data, but from the characteristic actions or operations in which the psychiatrist participates. The actions or operations from which psychiatric information is derived are events in interpersonal fields which include the psychiatrist” (1953, pp. 13–14).

Having come to the conclusion that schizophrenia has its origin in a disturbed interpersonal field, which the patient reproduces in his relationship with the psychiatrist in his role of “participant observer,” Sullivan centered his interpersonal theory around the way in which the interpersonal field influences and shapes the development of the individual.

On the theoretical level, Sullivan derived his field concept from Kurt Lewin’s (1890–1947) book *A dynamic theory of personality*, which he explicitly makes reference to in his *Interpersonal theory of psychiatry* (1953, p. 35). Indeed, this was also one of the theoretical sources of the work of Willy and Madeleine Baranger, who introduced the concept of field in the South American post-Kleinian tradition

at the beginning of the 1960s. Here is their own definition of the field concept:

The analytic situation should be formulated not only as a situation of one person who is confronted by an indefinite and neutral personage...but as a situation between two persons...involved in a single dynamic process...neither member of the couple can be understood without the other. (1961/2008, p. 796).¹

The fact that his interpersonal theory centers around not only the horizontal axis of the field concept, but also the vertical axis of development can be at best explained by making reference to how Sullivan himself talks about it in his *Interpersonal theory of psychiatry*. After having emphasized the necessity – originally formulated by W.A. White – of conceiving a psychiatry that not only “attempts to explain serious mental disorders,” but which can also be “of some use in living in general” (Sullivan, 1953, p. 4), here is how Sullivan introduced such a new developmental theory in the above-mentioned book:

How to communicate this particular theory of psychiatry has puzzled me and harassed me for a great many years, and I have finally come to the decision that the only approach is by the developmental route. In other words, if we go with almost microscopic care over how everybody comes to be what he is at chronologic adulthood, then perhaps we can learn a good deal of what is highly probable about living and difficulties in living. (1953, p. 4)

A central position in such a new theory is of course given by Sullivan (in a way similar to Melanie Klein) to the concept of anxiety, with particular regard to how it arises in the context of the interpersonal field. Here is how Sullivan goes about this:

Because a great many phenomena in the whole biological field are easier to understand if you trace them from their beginnings to their most complex manifestations, I would like to describe how I think anxiety begins in the infant. I do not know how early in life anxiety manifests itself. It is not exactly a field that you can get mothers and infants to cooperate in exploring...It is demonstrable that the human young in the first months of life...exhibits disturbed performance when the ‘mothering one’ has an emotional disturbance...Whatever the

infant was doing at the time will be interrupted or handicapped – that is, it will either stop, or will not progress as efficiently as before anxiety appeared. Thus anxiety is called out by emotional disturbances of certain types in the significant person – that is, the person with whom the person is doing something. (1953, pp. 8–9)

In other words, this is how Sullivan came to define the relational nature of human and mental development, that is, how he originally formulated what later became the basis of the intersubjective perspective. From another point of view, we can also emphasize the fact of how Sullivan’s new point of view, based on both observation and theoretical speculation, represented the dimension out of which Daniel Stern much later described what he called *The interpersonal world of the infant* (1985).²

Indeed, Sullivan’s theory of the interpersonal genesis of mind, development, and psychopathology was so powerful to allow him to theoretically formulate a series of consequences or principles that were only later taken up by authors such as Winnicott, Bion, and Kohut and eventually confirmed by empirical research. I am making reference to Winnicott’s concept that there is no baby without a mother, to Bion’s concept of maternal reverie, to Kohut’s concept of empathy, and to the empirical confirmation of Sullivan’s so-called “theorem of anxiety,” that is “*The tension of anxiety, when present in the mothering one, induces anxiety in the infant*” (1953, p. 41, original emphasis).³

At this point, before dealing with how Sullivan specifically articulates the therapeutic dimension of his theory in his posthumous book *The psychiatric interview* (1954), here is how he introduced it in the first chapter of his *Interpersonal theory of psychiatry*:

In attempting to outline this whole system of psychiatry, I want to stress from the very beginning the paralyzing power of anxiety. I believe that it is fairly safe to say that anybody and everybody devotes much of his lifetime, a great deal of his energy – talking loosely – and a good part of his effort in dealing with

¹ This connection with the work of Kurt Lewin did not escape Robert Hinshelwood in the paper he gave in Athens (Hinshelwood, 2010), in which he also emphasized Sullivan’s important role as pioneer of the intersubjective perspective. In this context, I can also add how exciting it was to hear in Athens also Madeleine Baranger herself, now 90 years old, give a paper on the pioneering work done with her husband Willy in South America.

² The important role played by Sullivan’s interpersonal theory in the development of his own research work was confirmed to me personally by Daniel Stern during the Athens Forum, at which he himself gave one of the key note papers.

³ I dealt with the many similarities between Sullivan’s and Bion’s life, personality, and work in my paper “Bion and Sullivan: An enlightening comparison” (2009). In a very good paper by the title “Klein, Bion, and intersubjectivity: Becoming, transforming, and dreaming” – which appeared in *Psychoanalytic Dialogues* during the preparation of this paper for publication – Lawrence Brown also emphasizes the interpersonal foundation of Bion’s concept of alpha function. He does this with the following words: “It is important to note that alpha function is the internalization of a complex intersubjective relationship between the mother and the infant and not just the internalization of a maternal function” (2010, p. 677, original emphasis).

others, to avoiding more anxiety than he already has and, if possible, to getting rid of his anxiety... For years and years psychiatrists have been struggling to cure this-and-that distortion of living as it came up in patients... But a much more practical psychotherapy seems to be possible when one seeks to find the basic vulnerabilities to anxiety in interpersonal relations, rather than to deal with symptoms called out by anxiety or to avoid anxiety. (Sullivan, 1953, p. 11)

Such a therapeutic approach centers around Sullivan's concept of participant observation, to which we now turn.

Sullivan and participant observation

"In the psychiatric interview a great part of the experience which one slowly gains," explains Sullivan in the first chapter of *The psychiatric interview*, "manifests itself in a show of mild interest in the point at which there is a tonal difference" (1954, p. 7). Thus we may conceive of the voice – rather than the words – as the "mirror of the patient's soul." This kind of listening and participation requires us to also ask very simple questions in order to clarify the meaning of the patient's words, for their meaning may deviate from the meaning we give them. "Thus part of the skill in interviewing," states Sullivan, "comes from a sort of quiet observation all along: 'Does this sentence, this statement, have an unquestionable meaning? Is there any certainty as to what this person means?'" (1954, p. 8). According to the concept of "consensual validation," efficient communication occurs only if we are able to agree on a common meaning after we have examined what meaning each of us gives to the words. Consensual validation, and its promotion through participant observation, forms the basis of Sullivan's concept of the interview – and of therapy.⁴

Of course, this is also the basis of Sullivan's approach to his patients' anxiety: anxiety is revealed through a change of the patient's voice; our sensitivity to it allows us to point this out to the patient; and equally important is our skill in helping the patient become aware of what makes him anxious. Participant observation allows us to be there and experience what the patient feels and, at the same time, to be detached enough from him to help him become aware of his feelings. In Sullivan's words:

Thus whenever the psychiatrist's attempt to discover what the patient is talking about leads the patient to be

somewhat more clear on what he is thinking about or attempting to communicate or conceal, his grasp on life is to some extent enhanced. And no one has grave difficulties in living if he has a very good grasp on what is happening to him. (1954, pp. 23–4)

Without going into the very articulated way in which Sullivan talks about his clinical work in this book, let us now limit ourselves to seeing how he specifically defines the concept of "participant observation," that is, his own conceptualization of what would later be called countertransference. The psychiatrist as participant observer has "an inescapable, inextricable involvement in all that goes on in the interview; and to the extent to which he is unconscious or unwitting of his participation in the interview, to that extent he does not know what is happening" (1954, p. 19). In other words, according to Sullivan, the therapist is on the one hand necessarily personally involved in what goes on in the session and, on the other hand, must become aware of it, for therapy to really take place.

As readers know, contemporary psychoanalysis has greatly expanded the space of the analyst's struggle with the process of becoming aware of what he is experiencing in the therapeutic relationship – as opposed to just making hypotheses on what is going on in the patient. From this point of view, Sullivan was both radical and conservative: without participation on the part of the analyst, no therapy takes place; and also, in order for therapy to take place, the analyst must be aware of all that happens in the relationship (and not just in the patient) – a view at variance with that of those contemporary analysts who see therapeutic change also occurring outside of the awareness of it.⁵ Indeed, conservative, that is, close to Freud, was Sullivan also in his application of the concept of participant observation to our relationship to the cultural and social factors that shape our personal identity. Following Freud, Sullivan had the kind of interdisciplinary orientation that contemporary psychoanalysis has lost. It is to this aspect of his work that we now turn.

Sullivan's interdisciplinary orientation

Chapter six of my book on Sullivan deals with "Sullivan as a social scientist." In it, I not only

⁴ It was also in connection with this attitude of Sullivan that Janet Rioch defined the interpersonal tradition he was the founder of as that tradition which "doesn't have answers, but only questions" (1969, p. 156). Not only Bion but also André Green has often underlined the importance of such a point of view.

⁵ A systematic comparison between Freud and Sullivan was articulated by Merton Gill in his famous paper "The interpersonal paradigm and the degree of the therapist's involvement" (1983). Although he had introduced the concept of the interpersonal field, Sullivan thought – as Freud also did – of the asymmetrical nature of the therapeutic relationship. From this point of view, the principle of co-construction of contemporary intersubjective analysts was merely hinted at, but not fully developed in his work.

show how his interpersonal point of view is rooted in the works of William James and George Herbert Mead and in the research projects of Chicago sociologists such as William Thomas and Robert Park, but also show how Sullivan himself “got his pants dirty doing field work” (to use an expression of Robert Park). Such work is documented in the anthology *The fusion of psychiatry and social science* that Helen Swick Perry edited in 1964 (Sullivan, 1964).

Here is, for example, what we can read in his “Discussion of the case of Warren Wall. A psychiatric gloss on a sociological study” (Sullivan, 1940/1964), that is, the report he wrote about a series of interviews he conducted with a black young man using his method of participant observation, in the context of a major research project on the condition of black people coordinated by the sociologist Franklin Frazier:

The tragedy of the Negro in America seems to be chiefly a matter of culturally determined attitudes in the whites, by the manifestations of which the Negro is generally distorted into a pattern of interracial behavior which permits the continuance of the attitudes without much change. This is a vicious circle, the interruption of which is an undertaking of great difficulty. (Sullivan, 1964, p. 106)

And here is Sullivan’s “prescription”:

It is evident that, if we are to develop a real approximation to national solidarity, we must find and cultivate a humanistic rather than a paternalistic, and exploiting, or an indifferent attitude to these numerous citizens of our commonwealth. As a psychiatrist, I have to speak particularly against using them as scapegoats for our unacceptable impulses; the fact that they are dark-skinned and poorly adapted to our historic puritanism is really too naïve a basis for projecting most of our privately condemned faults upon them. They deserve to be observed as they are, and the blot of an American interracial problem may then gradually be dissipated. (Sullivan, 1964, p. 107)

Of course, these words also show us how Sullivan had gone beyond Freud’s concept of the ego and had envisioned and conceptualized a self whose development depends not only on interpersonal, but also on social and cultural factors. Such a point of view has proved to be particularly useful in the globalized world in which we live, in which so many factors end up shaping our personal identity. On the other hand, even more than Freud, Sullivan connected his interdisciplinary orientation with a visible social commitment – in line with the critical attitude of Freud toward social issues, for example the repression of sexuality in the society of his time.

Concluding remarks

As I have tried to show in this paper, not only did Sullivan create the clinical, developmental, technical, and interdisciplinary premises that lie behind the present intersubjective perspective in psychoanalysis, but also the overall articulation of his work can still represent the best theoretical framework for it. If we go back to Jonathan Dunn’s definition of the intersubjective perspective, we can see how Sullivan himself pioneered a new concept of countertransference (through his concepts of participation and participant observation) and of ego function (through his concept of the interpersonal self). Last but not least, his point of view contributed – almost 50 years ago, in Amsterdam, 1962 – to the foundation of the International Federation of Psychoanalytic Societies, and keeping Sullivan’s point of view alive also helps us to keep the IFPS alive and well.

Let us go back to Sullivan and see what we can still learn from him. I have been dealing with his life and work for more than 20 years, and I still have the feeling that I keep profiting from his creative experience and insights. I hope that some of you can do the same.

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